[redacted]

FACSIMILE TRANSMITTAL COVER SHEET

CONFIDENTIAL HEALTH INFORMATION ENCLOSED

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Date [redacted] **Time:** _____ AM/PM

TO: Illumination foundation

Name of Authorized Recipient

Phone Number: ( ) _____

Fax Number: [redacted]

Number of pages (including cover sheet): _____

Street Address

City, State, Zip

FROM: Social Work Department

[redacted]

Name of Sender

[redacted]

Fax Number

[redacted]

TDD Telephone Number

Patient/Client Name: [redacted]

Information Faxed: _____

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ILLUMINATION FOUNDATION
RECUPERATIVE CARE

Exhibit B

LA/OC REFERRAL FORM

To refer a patient, please complete the entire form including supporting documents and **FAX** to [redacted] or **email** to [redacted]. For all referral questions, please contact IF Referral Coordinator at [redacted]

Referring Hospital: [redacted] **Dept/Floor:** [redacted]
SW/CM/RN: [redacted] **Phone/pager#:** [redacted]
E-mail: [redacted] **Nursing station#:**
Authorized by: [redacted] **phone#:** [redacted]

Patient Name: [redacted] **Date of Birth:** [redacted]

Gender: ☐ Male ☒ Female ☐ Transgender **Ethnicity:** _____ **Social Security# (last 4):** [redacted]

☐ **ED Visit** ☒ **Hospital Admit** **Date** [redacted] **Expected Discharge Date** [redacted]

Insurance Type: medical **English Speaking?** ☒ Yes ☐ No ***Primary Language Spoken:** _____

PLEASE VERIFY: ☒ **Currently homeless** **Check if:** ☐ **Homeless for more than 1 year continuously or 4 episodes in last 3 years**

CHIEF COMPLAINT/ADMITTING DIAGNOSIS:
left lower leg cellulitis cat bite, sepsis

SUBSTANCE ABUSE? ☒ None ☐ Alcohol ☐ Cocaine ☐ Heroin ☐ Meth ☐ Other: _____
Last Date Used: ___/___/_____ Current withdrawals: ☐ No ☐ Yes *Please explain_____

ANY WOUNDS? ☐ Yes ☒ No #/Loc/Size/Stage:_____

Independent with wound care? ☐ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☐ no

PPD/TB test or Chest XRAY performed? ☒ yes ☐ no Date: [redacted] Results: neg.

ANY LIMITATIONS, BEHAVIORAL CHALLENGES OR MENTAL HEALTH ISSUES? ☒ None ☐ Auditory/Visual Hallucinations

☒ Mental Health DX IF yes, **what type?:** depression, schizophrenia ☐ Non-compliant ☐ forgetful

☐ Cognitive Impairment ☐ Reg. Sex offender ☐ other, please explain _____

_____

Requires O2? ☐ Yes ☐ No Self Administer All Meds? ☒ Yes ☐ No *Please explain:_____

Continent of Bowel & Bladder? ☒ Yes ☐ No Colostomy/Ileostomy ☐ Yes ☒ No Foley Catheter ☐ Yes ☒ No

Independent w/all ADL's? ☒ Yes ☐ No *Please explain:_____

Diabetic? ☒ No if ☐ Yes, then: ☐ Insulin ☐ Oral Meds Communicable Diseases?:_____

Anticoagulants?: ☐ No IF ☐ yes, Requires INR/PT/PTT checks through Home Health or Clinic? _____

Ambulatory? ☒ Yes ☐ No Assistive device? ☐ No ☒ Yes *Please Explain: FWW

Does Patient have a car? ☐ Yes ☒ No Spouse/Partner? ☐ Yes ☒ No Service Animal? ☐ Yes ☒ No

Length of Stay authorized in Recuperative Care:____days

Attached Docs: ☒ **Face Sheet** ☒ **H&P** ☐ **Med List** ☐ **Psych notes** ☐ **Surgical notes** ☒ **PT/OT Eval.** ☒ **SW notes**

IF STAFF ONLY

Date Received:_____ Reviewed by:_____ Approved: ☐ Yes ☐ No Date:_____ Auth#_____

Denied Reason?:_____ Admission Date/time:_____ Exit Date:_____

Extension Requested?: ☐ no ☐ yes If yes, Approved additional days?:_____ by:_____ TOTAL LOS:_____

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[redacted]

Facesheet Report

[redacted]

Name: [redacted]		**Visit Type:**	Inpatient
OtherName:			
Address: [redacted]		**MRN:**	[redacted]
Phone (P): [redacted]		**Visit #:**	
Race: White	**Ethnicity:** Hispanic	**Location:**	[redacted]
Birthdate [redacted] 57y	**Notice Signed:** Yes-Version [redacted]	**Service:**	MEDICAL
Language: English	**Privacy Status:** NONE	**ED Admit:**	
Sex: Female	**Occurrence Code:** 11	**Admit Date/Tim**	[redacted]
[redacted] **Status:** Single	**Condition Code:**	**Accident Date:**	
Religion: Catholic, Roman	**Admit Source:** HOME OR NON HEALTH CARE FAC	**Mode Of Arrival:**	PRIVATE
Alert:	**Admit Type:** EMERGENCY	**Discharge Date/Time:**	
E-Mail [redacted]	**Discharge Disp:**		

GUARANTOR

Guarantor [redacted]	**ship:** Self	**Birthdate:**	[redacted]
Address:		**Phone (P):**	

RELATIVE

Relative: [redacted]	**Relationship:** Brother	**Phone (P)**	[redacted]
ddress: *, *, *, *, *, *		**Phone (W):**	

PHYSICIANS

Admitting: [redacted]		Phone (W):	[redacted]
Attending:		Phone (W):	

INSURANCE

Plan Code: [redacted]	**Relationship:** Self	**Insured Nam**	[redacted]
Carrier/Plan: [redacted]			
Address		**Policy**	[redacted]
Comment: CAT BITE FROM [redacted] FROM STEPPING ON CAT. Phone#: [redacted]		**Authorization**	

INSURANCE

Plan Code: [redacted]	**Relationship:** Self	**Insured Name**	[redacted]
Carrier/Plan: [redacted]			
Address *, *, *, *, *, *		**Policy #:**	
Comment:		**Authorization:**	

DIAGNOSIS

Visit Reason: LEFT LOWER LEG CELLULITIS CAT BITE, [redacted]

Admit Clerk: [redacted]

Note :

COT Completed:	**COT Completed Date:**

[redacted] **CONFIDENTIAL PATIENT INFORMATION** [redacted]

Chief Complaint:

- Left lower extremity swelling.

History of Present Illness:

- 67-year-old female with a history of hyperlipidemia, depression and schizophrenia presented to the emergency room complaining of left lower extremity swelling for the past 2 days. Patient is lethargic and poor historian. Most history is obtained from ER physician's account with patient. Patient has cat bite 2 days ago worse worsening left lower extremities redness and swelling and tenderness. Patient denies fever or chill. Patient also complaining of progressive worsening shortness of breath with ambulation for the past one month. Denied cough or congestion. Patient denies history of cardiac or pulmonary disease.

Past Medical History:

- Hyperlipidemia

Depression

Schizophrenia.

Past Surgical History:

- Unknown.

Medications:

Patient Currently Takes** [redacted) **ocumented in Structured Notes

Allergies:

No Known Allergies: Active

Social History:

- **Habits:** Denied tobacco or alcohol abuse
- **Living Situation and Functional Status:** Unknown

Family History:

- Unknown.

Review of Systems:

• Denies fevers or chill
Denies headache or blurred vision
Denies chest pain or palpitation
Denies shortness of breath or dyspnea exertion
Denies abdominal pain, nausea vomiting diarrhea constipation
Denies melena or hematochezia
Denies dysuria or hematuria
Denies skin rash.

Vitals:

• **Vitals:**
Temp: 100.8 (Tmax 100.8) **HR:** 89 **BP:** 120/74 pulse pressure: 46 **RR:** 23 **SpO2:** 97% on 2L via nasal cannula

Estimated PF ratio: 392
BMI: 39.5

• **Physical Exam::** General. Arousable but lethargic, in no acute distress, obese
Head. Normocephalic and traumatic
EENT. Pupils equal round and reactive to light, extraocular muscles intact
CV, regular rate rhythm, no murmur no gallop no rub
Lung. Clear to auscultation bilaterally
Abdomen. Soft, nondistended, nontender to palpation, no rebound or guarding
Extremity. Left lower thigh leg with 2 areas of puncture wounds with no evidence of abscess with diffuse erythema poorly [redacted] along the anterior shin and left lateral leg with notable swelling,
No evidence of crepitus on examination. No joint involvement
Neuro, Unable to follow commands cranial [redacted] grossly intact
Skin. No rash

Laboratory:

• **Details:**

BNP

[redacted]	B-Type Natriuretic Peptide (BNP)
	60

BMP

	NA	K	Cl	CO2	Gln	BUN	Cr	Ca

[redacted]

[redacted] 137 [redacted] 102 26 99 8 [redacted]

CBC

	WBC	Hgb	Plt	HCT
[redacted]	[redacted]	[redacted]	269	[redacted]

CRP

	CRP
[redacted]	[redacted]

Troponin

	Troponin I HS
[redacted]	6

Lactic Acid

	Lactic Acid
[redacted]	0.8

Radiology:

Select days to review: 2

CT Left Lower Ext with IV Contrast [redacted]

IMPRESSION:

DIFFUSE CELLULITIS THROUGHOUT THE LEFT LOWER THIGH, KNEE, LEG, ANKLE, AND FOOT

NO ABSCESS, FASCIAL EDEMA, OR SOFT TISSUE GAS

Venous Doppler Lower Ext Bilateral [redacted]

IMPRESSION:

THERE IS NO EVIDENCE OF DEEP VEIN THROMBOSIS IN THE VISUALIZED VEINS OF BILATERAL LOWER EXTREMITIES.

HOWEVER THE BILATERAL PERONEAL VEINS AND THE RIGHT DISTAL FEMORAL VEINS COULD NOT BE VISUALIZED AND HENCE CANNOT RULE OUT DVT IN THESEVEINS.

TECHNICAL DIFFICULT STUDY.

Chest XR 1-View [redacted]

IMPRESSION:

No acute pulmonary disease.

[redacted]

[redacted]

• .:
#Left lower extremity cellulitis secondary to cat bite
#Sepsis secondary to cellulitis
- Admit to MedSurg unit
- Await blood culture
- Continue empiric IV antibiotics with vancomycin and Unasyn
- Continue supportive care
- Pain control with IV and p.o. narcotic

#Dyspnea on exertion, unclear etiology
- Chest x-ray and BNP are normal
- Will obtain echocardiogram to rule out pulmonary hypertension

#Obesity. Recommend lifestyle modification

#Hyperlipidemia. Continue statin

History of schizophrenia. Continue olanzapine and Celexa

#GI prophylaxis Pepcid, DVT prophylaxis with heparin

Impression/Problem List:

1. **Sepsis:**

CPT Codes:

[redacted]

[redacted]

Documents Review Report

[redacted]

• CPT Codes [redacted]

Electronic Signatures:

[redacted]

Entered: *Chief Complaint, History of Present Illness, Past Medical History, Past Surgical History, Allergies, Social History, Family History, Review of Systems, Vital Signs, Physical Exam, Laboratory, Radiology, Assessment and Plan, CPT Billing*

Authored: *Chief Complaint, History of Present Illness, Past Medical History, Past Surgical History, Medications, Allergies, Social History, Family History, Review of Systems, Vital Signs, Physical Exam, Laboratory, Radiology, Assessment and Plan, CPT Billing*

Last Updated [redacted]

[redacted]

[redacted]

Documents Review Report

[redacted]

[redacted] **Occupational Therapy Initial Evaluation** [redacted] ****Revised****

Prior Level of Function:
Previous Level of Function:

- **Living Arrangements:** homeless
- **Bathing::** Always Independent
- **Upper Body Dressing::** Always Independent
- **Lower Body Dressing::** Always Independent
- **Toilet Hygiene::** Always Independent
- **Assistive Devices Used:** Independent with all self-cares.

General Information:

- **Onset of Illness/Injury (mmm-dd-yy)** [redacted]
- **Diagnosis:** Left lower leg cellulitis from cat bite, [redacted]
- **MD Orders:** Occupational Therapy Evaluation
- **Referring Physician** [redacted]
- **Pertinent History of Current Problem:**

Patient is a 67 female who presents with left hip pain, warmth to the left lower leg, and worsening swelling after a stray cat bit her 2 days ago. PMH(x): hyperlipidemia, depression, schizophrenia,

Pain:

- **Location of Pain:** [redacted]

Vital Signs:
Respiratory:

- **SpO2 % with activity:** 95 percent
- **Oxygen Device:** nasal cannula

Bed Mobility:

- **Bed Mobility Training: Supine-to-Sit:** independent
- **Bed Mobility Training: Sit-to-Supine:** independent

Transfer Training:

- **Transfer Training: Sit-to-Stand Transfer:** independent, front wheel walker

Walking:

- **Walking:** independent front wheel walker

[redacted]

[redacted]

Toilet Transfer/Hygiene:
Toilet Transfer:

Transfer Skill: Toilet Transfer
• **Transfer Training:** independent

Toilet Hygiene Skills:

Toilet Hygiene Skills:
• **Toilet Training:** independent

Dressing Skills:
Upper Body Dressing Skills:

Upper Body Dressing Skills:
• **Upper Body Dressing Training:** independent

Lower Body Dressing Skills:

Lower Body Dressing Skills:
• **Lower Body Dressing Training:** independent, does not wear socks at baseline

Grooming/Hygiene Skills:
Grooming Skills:

Grooming Skills:
• **Grooming Training:** independent

Eating/Self Feeding Skills:
Eating/Self-Feeding Skills:

Eating/Self-Feeding Skills:
• **Level of Independence:** independent

Balance Assessment:
• **Balance Skills: Sitting Balance:** normal balance
• **Balance Skills: Standing Balance:** good balance w/ FWW

Education:
• **Rehabilitation:** safety with ADL's, energy conservation/pursed lip breathing, patient instructed;

[redacted]

verbal instruction; verbalized/acknowledged understanding, (2) meets goals/outcomes

Clinical Impression:

- **Criteria for Skilled Therapeutic Interventions Met:** does not meet criteria for skilled intervention

Assessment:

- **Safety:** patient appears safe to walk with nursing..., patient requires assistance to prevent a fall...
- **Assessment Detail:** Agreeable. Slightly impulsive although no LOB throughout. Patient demonstrates strength and endurance consistent with baseline. No new impairments noted that would limit ability to complete self-care or functional tasks at prior level of performance.

OT Recommendation:

- Patient is independent for all self care and does not require further OT intervention.

Recommendations:

- **Anticipated Discharge Disposition (OT):** back to Illumination Foundation

Safety / Fall Interventions:

- **WDL Moderate Risk Fall Interventions** all interventions implemented [redacted] **[bed/wheels in lowest position and locked, call light/personal items in close proximity, patient education, red footwear, side rails up x 3, yellow wristband, ambulate per MD Orders and ALWAYS [redacted] staff/students, gait belt with all mobility, within arms reach during ambulation/toileting/transfers]**

Notification:

- **Person Notified** [redacted] Discharge from occupational therapy, eval outcome

Evaluation Complexity:

Evaluation Complexity:

- **Evaluation Complexity:** 1 - 2 Factors [redacted] Elements Stable and/or Uncomplicated

- .: Low Complexity

Closing Check Off:

- **Evaluation Duration:** 60 minutes
- **Occupational Therapy Total Treatment Time:** 0 minute(s)

Electronic Signatures:

[redacted]

Authored: *Prior Level of Function, General Information, Pain, Vital Signs, Bed Mobility, Transfer Training, Walking, Toilet Transfer/Hygiene, Dressing Skills, Grooming/Hygiene Skills, Eating/Self Feeding Skills, Balance Skills Assessment, Education, Clinical Impression, Assessment, OT Recommendation, Recommendations, Falls / Safety Interventions, Notification, Evaluation Complexity, Closing Check Off*

Last Updated [redacted]

Notification

SBAR Notification
Person Notified Person Not[redacted] Discharge from occupational therapy, eval outcome
(S) Situation or Critical Results (S) Situation or Critical Results: PT evaluation outcome, DC recommendations

Therapeutic Interventions

Bed Mobility
Bed Mobility Rolling/Scooting Bed Mobility Rolling/Scooting: independent
Bed Mobility Training: Supine-to-Sit Supine-to-Sit: independent
Bed Mobility Training: Sit-to-Supine Sit-to-Supine: independent

Sit-Stand Transfer Training
Transfer Training: Sit-to-Stand Transfer Sit-to-Stand Transfer: independent front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker

Gait Training
Gait Training Gait Training: independent front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker; front wheel walker
Gait Analysis Gait Analysis: step-through gait steady gait

Weight Bearing Restrictions: weight-bearing as tolerated
Balance Skills
Balance Skills: Sitting Balance Balance Skills: Sitting Balance: normal balance
Balance Skills: Standing Balance Balance Skills: Standing Balance: good balance w/ FWW

Grooming Training
Grooming Training Assistance: independent

Upper Body Dressing Training
Upper Body Dressing Training Assistance: independent

Lower Body Dressing Training
Lower Body Dressing Training Assistance: independent does not wear socks at baseline

Ambulation
Walking Walking: independent front wheel walker

Toilet Transfer Training
Transfer Training Toilet Transfer: independent

Toilet Training
Toilet Training Assistance: independent

Physical Therapy Treatment Time minute(s): 0 minute(s)
Occupational Therapy Total Treatment Time minute(s): 0 minute(s)
Assessment
Safety Safety: patient appears safe to walk with nursing.... patient requires assistance to prevent a fall...
Assessment Detail Detail: Agreeable. Slightly impulsive although no LOB throughout. Patient demonstrates strength and endurance consistent with baseline. No new impairments noted that would limit ability to complete self-care or functional tasks at prior level of performance.

[redacted]

Documents Review Report

[redacted]

[redacted] **Physical Therapy Initial** [redacted]
Evaluation

Prior Level of Function:
Previous Level of Function:

- **Living Environment/Prior Level of Functioning Reviewed/Completed on Patient Profile:** Yes
- **Lives With:** alone
- **Living Arrangements:** resides at the Illumination Foundation
- **Transferring:** Always Independent
- **Ambulation:** Always Independent
- **Ambulation Distance:** community
- **Assistive Devices Used:** pt denies owning any DME

General Information:

- **Onset of Illness/Injury (mmm-dd-yy)** [redacted]
- **Diagnosis:** LLE cellulitis, [redacted]
- **MD Orders:** PT Evaluation
- **Weight Bearing Restrictions:** weight-bearing as tolerated
- **Referring Physician** [redacted]

Pertinent History of Current Problem:
Pt admitted to ED for LLE worsening swelling and redness after being bit by a stray cat. PMHx: HLD, depression, schizophrenia

- **Test Results::** CT Angio Heart: Multifocal calcified plaque at the left circumflex artery causing moderate [redacted] stenosis at the mid aspect. Multifocal calcified plaque at the LAD and RCA with mild stenoses; CT Head: (-) acute changes, Venous Doppler BLE: (-) for DVT in visualized veins, CT LLE: DIFFUSE CELLULITIS THROUGHOUT THE LEFT LOWER THIGH, KNEE, LEG, ANKLE, AND FOOT; CT Angiogram Chest: (-) for PE
- **General Precautions/Limitations:** falls; bed alarms
- **Equipment:** IV

Pain:

- **Pain Rating::** 0

Vital Signs:
Respiratory:

- **SpO2 % with activity:** 92 percent
- **Oxygen (L/min):** 1 L/min
- **Oxygen Device:** nasal cannula

[redacted]

[redacted]

Range Of Motion Examination:
- **Range of Motion Examination:** BLE ROM is WFL

Manual Muscle Testing:
- **Manual Muscle Test Exam:** B LE strength is WFL

Bed Mobility:
- **Bed Mobility Rolling/Scooting:** independent
- **Bed Mobility Training: Supine-to-Sit:** independent
- **Bed Mobility Training: Sit-to-Supine:** independent

Transfer Training:
- **Transfer Training: Sit-to-Stand Transfer:** independent, front wheel walker

Gait Training:
- **Gait Training:** independent, front wheel walker
- **Gait Analysis:** step-through gait steady gait

Balance Skills Assessment:
- **Balance Skills: Sitting Balance:** good balance
- **Balance Skills: Standing Balance:** good balance

Education:
- **Rehabilitation:** bed mobility techniques, transfer techniques, gait skills, use of assistive device/adaptive equipment, patient instructed; family not present; verbal instruction; verbalized/acknowledged understanding, returned skill demonstration, (2) meets goals/outcomes

Clinical Impression:
- **Criteria for Skilled Therapeutic Interventions Met:** no; does not meet criteria for skilled intervention

Assessment:
- **Safety:** patient appears safe to walk with nursing..., patient appears safe to transfer to

[redacted]

chair/BSC/WC with nsg..., with FWW and gait belt, patient requires assistance to prevent a fall...

- **Response to Treatment:** motivated...
- **Assessment Detail:** Pt received in bed, agreed to PT evaluation. Pt able to demonstrate independent mobility. Steady gait with FWW, no LOB noted. No skilled PT services indicated as pt is grossly at functional baseline.

PT Recommendation:

- Patient is independent for all functional mobility and does not require further PT intervention.

Outcome Goals:

Patient/Family Goal: feel better.

Planned Therapy Interventions:

Plan discussed with: patient

Recommendations:

- **Anticipated Equipment Needs at Discharge (PT):** front wheeled walker; The patient has a mobility limitation that significantly impairs the patient's ability to participate in mobility related IDLs. The functional mobility deficit will be sufficiently resolved with the use of the walker. The patient is able to safely use the walker.
- **Anticipated Discharge Disposition (PT):** back to Illumination Foundation

Safety / Fall Interventions:

• **WDL Moderate Risk Fall Interventions [redacted] [bed/wheels in lowest position and locked, call light/personal items in close proximity, patient education, red footwear, side rails up x 3, yellow wristband, ambulate per MD Orders and ALWAYS [redacted] staff/students, gait belt with all mobility, within arms reach during ambulation/toileting/transfers]**	all interventions implemented

Notification:

- **Person Notified** [redacted]
- **(S) Situation or Critical Results:** PT evaluation outcome, DC recommendations

Treatment Plan:

- **Next Treatment Plan** DC from PT services

Evaluation Complexity:

Evaluation Complexity:

- **Evaluation Complexity:** 0 Factors [redacted] Elements Stable and/or Uncomplicated
- .: Low Complexity

Closing Check Off:

- **Evaluation Duration:** 60 minutes
- **Physical Therapy Treatment Time:** 0 minute(s)

Electronic Signatures: [redacted]

Authored: *Prior Level of Function, General Information, Pain, Vital Signs, Range Of Motion Examination, Manual Muscle Testing, Bed Mobility, Transfer Training, Gait Training, Balance Skills Assessment, Education, Clinical Impression, Assessment, PT Recommendation, Outcome Goals, Planned Therapy Interventions, Recommendations, Falls / Safety Interventions, Notification, Treatment Plan, Evaluation Complexity, Closing Check Off*

Last Updated [redacted]

[redacted]

[redacted]

Patient is a 67 year old Female whose length of stay is 6 days.

- **Subjective ::**

gen weak/HA improving
at times fluctuating mental status per d/w RN, though today appears to be less confused and AAOx2-3 to [redacted]ays her DOB [redacted]date today
leg swelling/redness improving

- **Vitals:**

Temp: [redacted](Tmax 98) **HR:** 55 **BP:** 110/60 pulse pressure: 50 **RR:** 18 **SpO2:** 98% on 1L via medical device removed/checked for skin/mucosal assessment
BMI: [redacted]

In: 860 **Out:** 1850 **Net:** -990 (24 Hour Total)

- **Physical Exam::** · General Appearance: well developed, well nourished, obese, female, no acute distress, lying in bed, minimally cooperative with exam
  · ENT: moist mucous membranes
  · Respiratory: respiratory effort normal at rest, did not let examine from back
  · CVS: regular rate and rhythm
  · Abdomen/GI: soft, no obvious tender, bowel sounds present
  · M-S/Extremities: LLE swelling/redness improving compared to [redacted]area
  · Neurologic: moving ext, did not cooperate with further exam
· Psychiatric: as above

Lab:

- **Details:**

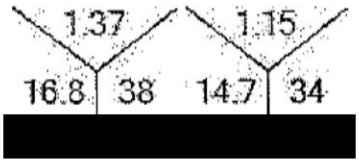
142	100	10	97	143	100	9	89	142	99	11	88
4.3	33	0.76		4.1	34	0.79		3.9	35	0.84	

[redacted]

7.9	10.7	269		8.5	11.1	267		8.9	11.8	299	
[redacted]											

[redacted]

[redacted]



[redacted]

CRP

[redacted] CRP [redacted]

LFT

[redacted]

	ALT	AST	ALP	TBili	Protein	Alb	DBili
[redacted]	16	15	[redacted]	[redacted]	[redacted]	[redacted]	
	20	15	132	[redacted]	[redacted]	[redacted]	[redacted]
	16	16	122	[redacted]	[redacted]	[redacted]	[redacted]

Hgb A1c

[redacted] Hemoglobin A1c % [redacted]

TSH and FT4

	TSH	Free T4
[redacted]	0.934	[redacted]

Cultures - Entire Hospitalization:

[redacted] Culture, Blood 1st Draw
Report Stat [redacted]
Specimen Description Venous Blood
Culture No growth after 5 days
Special Requests None

[redacted] Culture, Blood 2nd Draw
Culture No growth after 5 days
Report Stat [redacted]
Special Requests None
Specimen Description Venous Blood

[redacted] Urinalysis & Culture If Indicated
Color Yellow
Appearance TURBID
Glucose, Urine NORMAL
Ketones Trace
Bilirubin, Urine Negative
Blood Negative
Nitrite Negative
Esterase, Urine Negative

[redacted]

[redacted]

Squamous Epith Many
WBCs / HPF [redacted]
RBC's Urine [redacted]
Bacteria Urine Rare
Culture if indicated CULTURE INDICATED
Amorphous Crystals Rare
TDS MCOLU Straight cath

[redacted] Culture, Urine
Report Statu[redacted]
Specimen Description Straight cath
Culture <10,000 CFU/mL Normal Urogenital Flora
Special Requests None

Radiology and Other Studies:
• **Select days to review:** 2
• .:
CT Head no IV Contras [redacted]
IMPRESSION:
No acute intracranial hemorrhage or mass effect.

• .:
Active Medications

No Category:
• Anticoagulation Monitoring Per Pharmacy: 1 each Rx Protocol daily
anti-infectives:
• ampicillin-sulbactam IVPB: 3 gram IVPB every 6 hours
cardiovascular agents:
• furosemide: 40 milligram oral daily (LASIX)
• metoprolol tartrate: 25 milligram oral 2 times daily (LOPRESSOR) 25 milligram oral 2 times daily
central nervous system agents:
• aspirin Tablet Enteric Coated: 81 milligram oral daily (ECOTRIN)
• acetaminophen Tablet PRN: 650 milligram oral every 4 hours (TYLENOL)
• HYDROcodone-acetaminophen 5-325 mg PRN: 1 tablet(s) oral every 4 hours (NORCO 5-325)
• LORazepam PRN: [redacted] milligram oral 3 times daily (ATIVAN)
• morphine Injection PRN: 2 milligram IV every 2 hours (morphine)
• ondansetron Injection PRN: 4 milligram IV every 6 hours (ZOFRAN)
coagulation modifiers:
• heparin Injection: 5000 unit(s) subcutaneous.. every 12 hours
gastrointestinal agents:
• famotidine: 40 milligram oral daily (PEPCID)
immunologic agents:
[redacted]

[redacted]

• influenza vaccine Injection - Adult (65 and older): [redacted] milliliter intramuscular One Time
metabolic agents:
• atorvastatin: 10 milligram oral daily at bedtime (LIPITOR)
psychotherapeutic agents:
• citalopram: 10 milligram oral daily (CeleXA)
• OLANZapine: [redacted] milligram oral daily at bedtime (ZyPREXA)
• OLANZapine Injection PRN: 10 milligram intramuscular One Time (ZyPREXA)

Impression:

Problem List:

1. **Cellulitis, leg**:
2. **Sepsis**:
3. **CRP elevated**:
4. **Leukocytosis**:
5. **Acute metabolic encephalopathy**:
6. **Morbid obesity**: Description: Morbid obesity
7. **Bradycardia**:

Problem List - Chronic:

History of alcohol use:
Current smoker:
History of methamphetamine abuse:
Homelessness:
Dyslipidemia:
History of suicidal ideation:
Depression:
Morbid obesity:

• .:
LLE cellulitis, s/p cat scratch per reports
abx - unasyn
monitor wbc, crp, temp
monitor vitals, hemodynamic status

dyspnea, acute hypercapneic respiratory failure - likely multifactorial
PFT, CTA etc. as above
echo prelim EF ok, DD
CTA coronary per cardio as above
continue bipap when napping and QHS for probable OSA - patient has not been tolerating bipap

[redacted]

[redacted]

patient will need outpatient sleep study, pt agreed with plan
HR a bit lower side, [redacted] BB to bid, monitor vitals, hemodynamic status and adjust med prn - consider [redacted] dose to 12.5mg bid
f/u cardio/pulm recommendations

?meth/etoh/tobacco use?, ?schizo/?depression/?mental health issues etc. per reports - social work consult
continue zyprexa and celexa
obesity - life style modifications advised
PT
HA - CT head as above, fluctuating mental status, monitor mental status, c/w reorient, improving compared to yesterday

d/w pt, consults, RN and all agreed with above, all questions answered.

CPT Codes:
- **CPT Codes** [redacted]

Electronic Signatures:
[redacted]

Authored: *Subjective, Vitals, Physical Examination, Lab, Radiology and Other Studies, Medications, Impression, Additional Assessment/Plan, Billing*

Last Upda [redacted]

[redacted]

[redacted]

[redacted]

• **Subjective ::**
No new cardiac issues.

• **Vitals:**
Temp: [redacted] (Tmax [redacted] **HR:** 56 **BP:** 104/66 pulse pressure: 38 **RR:** 19 **SpO2:** 98% on 1L via nasal cannula
BMI: [redacted]

In: 860 **Out:** 1850 **Net:** -990 (24 Hour Total)

• **Physical Exam::** GENERAL: In no acute distress, well developed, well nourished
HEENT is unremarkable. There is no scleral icterus. No conjunctivitis. Ears no discharge. Tongue is smooth and papillated.neck is supple. No jugular venous distention. Trachea central. Carotids are palpable.
Chest is bilaterally symmetrical. Lungs are clinically clear.
Cardiovascular. PMI is not visible. Examination the heart reveals normal heart sounds. No murmurs or gallops. No pericardial friction rub.
Carotids are palpable. No bruit. Abdominal aorta is not palpable. Femoral pulses are felt bilaterally. No bruit. Dorsalis pedis and posterior tibial not palpable.
Extremities. No clubbing, or cyanosis. No edema.
Neurological no focal or gross deficits.
Psychiatric. Alert and oriented.
Abdomen. Nontender. No rigidity. Bowel sounds intact. No hepatosplenomegaly, no masses palpable.
Genitalia and rectal deferred.
Musculoskeletal: spine: no kyphoscoliosis. Gait: normal. Muscle: normal muscle tone, no weakness
Skin: no rashes, no nodule

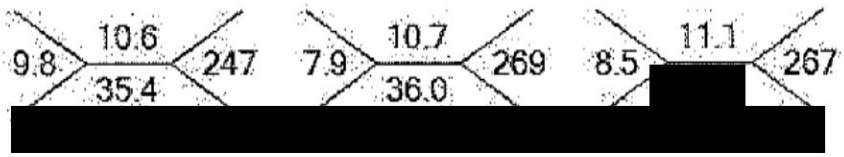
Lab:
• **Details:**

139	100	9	109	142	100	10	97	143	100	9	89
4.1	30	0.80		4.3	33	0.76		4.1	34	0.79	

[redacted]

[redacted]

[redacted]



[redacted]

Troponin

	Troponin I HS
[redacted]	6

Radiology and Other Studies:

- **Select days to review:** 1000
- .:

CT Head no IV Contras [redacted]

IMPRESSION:

No acute intracranial hemorrhage or mass effect.

CT Angio Heart w/wo IV Contrast (Coronary Arteries) [redacted]

Impression:

1. Multifocal calcified plaque at the left circumflex artery causing moderate [redacted] stenosis at the mid aspect.
2. Multifocal calcified plaque at the LAD and RCA with mild stenoses.
3. Calcium score of 735.2.

This corresponds to a 97 percentile ranking for patient's age and sex.

The percentile ranking compares the calcium score with scores for participants in the multiethnic study of atherosclerosis (MESA) of the same gender and similar age that were free of cardiovascular disease. A percentile ranking below 50% indicates a score better than most in the group. A percentile ranking above 50% indicates a score worse than most in that group.

Knee XR Left 4-Views - [redacted]

IMPRESSION:

1. SEVERE CHANGES OF TRICOMPARTMENTAL OSTEOARTHRITIS.
2. SMALL JOINT EFFUSION AND SOFT TISSUE SWELLING.

CT Angio Chest with IV Contrast [redacted]

IMPRESSION:

1. No pulmonary embolism.
2. Main pulmonary artery diameter measures 3.1cm in diameter, as can be seen with pulmonary hypertension.
3. Trace bilateral pleural effusions and mild lateral posterior dependent atelectasis.

Lower Extremity Arterial Doppler Bilatera [redacted]

IMPRESSION:

[redacted]

[redacted]

ABNORMAL BILATERAL LOWER EXTREMITY ARTERIAL NON-INVASIVE VASCULAR EXAM CONSISTENT WITH THE FOLLOWING:
RIGHT LOWEREXTREMITY DORSALIS PEDIS ARTERY MONOPHASIC WAVEFORMS CONSISTENT WITH ARTERIAL DISEASE IN THIS VESSEL.
NO EVIDENCE OF ANY HEMODYNAMICALLY SIGNIFICANT STENOSIS OR FLOW OCCLUSION IN ANY OF THE VISUALIZED ARTERIES.
UNABLE TO CLEARLY VISUALIZE THE LEFT LOWER EXTREMITY PERONEAL ARTERY AND THEREFORE CANNOT EXCLUDE STENOSIS OR OCCLUSION IN THIS VESSEL.
TOE PPG STUDIES DEMONSTRATED WAVEFORMS BILATERALLY, WITH THE RIGHT SIDE BEING SOMEWHAT MORE DAMPENED.
TECHNICALLY DIFFICULT STUDY.

Upper Ext Arterial Dopple[redacted]
IMPRESSION:
NORMAL BILATERAL UPPER EXTREMITY ARTERIAL DUPLEX DOPPLER EXAM.

Echocardiogram[redacted]

CT Left Lower Ext with IV Contras[redacted]
IMPRESSION:
DIFFUSE CELLULITIS THROUGHOUT THE LEFT LOWER THIGH, KNEE, LEG, ANKLE, AND FOOT
NO ABSCESS, FASCIAL EDEMA, OR SOFT TISSUE GAS

Venous Doppler Lower Ext Bilatera[redacted]
IMPRESSION:
THERE IS NO EVIDENCE OF DEEP VEIN THROMBOSIS IN THE VISUALIZED VEINS OF BILATERAL LOWER EXTREMITIES. HOWEVER THE BILATERAL PERONEAL VEINS AND THE RIGHT DISTAL FEMORAL VEINS COULD NOT BE VISUALIZED AND HENCE CANNOT RULE OUT DVT IN THESEVEINS.
TECHNICAL DIFFICULT STUDY.

Chest XR 1-View[redacted]
IMPRESSION:
No acute pulmonary disease.

• .:
Active Medications

- acetaminophen Tablet PRN: 650 milligram oral every 4 hours (TYLENOL)
- ampicillin-sulbactam IVPB: 3 gram IVPB every 6 hours
- Anticoagulation Monitoring Per Pharmacy: 1 each Rx Protocol daily

[redacted]

[redacted]

- aspirin Tablet Enteric Coated: 81 milligram oral daily (ECOTRIN)
- atorvastatin: 10 milligram oral daily at bedtime (LIPITOR)
- citalopram: 10 milligram oral daily (CeleXA)
- famotidine: 40 milligram oral daily (PEPCID)
- furosemide: 40 milligram oral daily (LASIX)
- heparin Injection: 5000 unit(s) subcutaneous.. every 12 hours
- HYDROcodone-acetaminophen 5-325 mg PRN: 1 tablet(s) oral every 4 hours (NORCO 5-325)
- influenza vaccine Injection - Adult (65 and older): [redacted] milliliter intramuscular One Time
- LORazepam PRN: [redacted] milligram oral 3 times daily (ATIVAN)
- metoprolol tartrate: 25 milligram oral 2 times daily (LOPRESSOR) 25 milligram oral 2 times daily
- morphine Injection PRN: 2 milligram IV every 2 hours (morphine)
- OLANZapine: [redacted] milligram oral daily at bedtime (ZyPREXA)
- OLANZapine Injection PRN: 10 milligram intramuscular One Time (ZyPREXA)
- ondansetron Injection PRN: 4 milligram IV every 6 hours (ZOFRAN)

Impression:
Problem List:

1. **Chest pain**:
2. **Methamphetamine use**:
3. **Restrictive lung disease**:
4. **Acute coronary syndrome**:
5. **Acute metabolic encephalopathy**:
6. **Obstructive sleep apnea**:
7. **Acute heart failure with preserved ejection fraction (HFpEF)**:
8. **Diastolic dysfunction**:

Problem List - Chronic:

History of alcohol use:
Current smoker:
History of methamphetamine abuse:
Homelessness:
Dyslipidemia:
History of suicidal ideation:
Depression:
Morbid obesity:

- .:

He was

[redacted]

[redacted]

[redacted]

I am asking for a whole family and then you are supposed to g[redacted]
67-year-old lady with history of hyperlipidemia, obstructive sleep apnea, diastolic dysfunction, heart failure with preserved ejection fraction, presents with chest pain while in the hospital. She has symptoms suggestive of acute coronary syndrome. Given her clinical profile a pharmacological stress test was recommended but the patient[redacted] this. She has agreed to coronary CT angiographic study. She has been complaining of dyspnea on exertion and has evidence of diastolic dysfunction. She has heart failure with preserved ejection fraction. In addition she has cellulitis of her lower extremities again post cat scratch disease.

Empiric IV antibiotics for[redacted] Chest x-ray and BNP are unremarkable.
Given her history of chest pains while in the hospital elevated troponin, CT coronary angiographic study. Hyperlipidemia be treated with statin therapy. Troponins will be trended. ACS pathway will be instituted.

[redacted]
No new chest pain.
CTA SHOWS 50 T0 60% LCX SENOSIS
RECOMMEND MEDICAL MANAGEMENT
HEART FAILURE WILL BE TREATED WITH FUROSEMIDE AS NEEDED
Hyperlipidemia will be treated with statin

[redacted]No new cardiac issues. Chest pain resolved. Coronary CT angiogram shows intermediate severity of CAD in the circumflex artery. No further cardiac investigation needed. CHF will be treated with furosemide. Recommend statin therapy for hyperlipidemia. Patient on metoprolol tart rate and this can be switched to metoprolol succinate because of CHF

CPT Codes:
- **CPT Codes** [redacted]

Electronic Signatures:
[redacted]

Authored: *Subjective, Vitals, Physical Examination, Lab, Radiology and Other Studies, Medications, Impression, Additional Assessment/Plan, Billing*

Last Updated [redacted]

[redacted]

[redacted]

[redacted]

Patie[redacted]
MRN
Acco
Date
Care Management Initial Adult Assessment
Advanced Care Planning
Assessed Patient/Family readiness, willingness, and ability to engage or provide support in self-management activities" Yes
Designated Decision-Maker Same as Facesheet Emergency Contact
Mental Status Exam
Alert and Oriented Oriented x 4
Behavior No Eye Contact
Speech Mumbled
Affect Agitated/Irritable
Appearance Well Kempt
Thought Content Appropriate
Thought Process Linear and Goal Directed
Insight Fair
Judgment Fair
Coping
Coping Within Normal Limits
Prior Level of Function
Living Condition Homeless
Notes: LCSW referred to pt for homeless. Pt was laying in bed with her eyes closed. Pt states she is from Illumination foundation and came into hospital due to cat bite. LCSW called Illumination foundation and spoke with[redacted] states pt was recently there and is not a DHS pt therefore needs a new auth from me[redacted]. Pt has hx of depression, schizophrenia. Per pt, she takes her psych medication. RN was doing assessment and states pt was alert earlier in the morning. LCSW to follow up with pt when more awake as pt is refusing Bipap, as pt states she does not like it on.
Ambulation Independent
ADL (Grooming, Dressing, Bathing, Feeding) Independent
IADL (Medication Management, Cooking, Cleaning, Paying Bills) Independent
Home DME Front Wheel Walker
Readmission Interventions
Education Provided Care Management Department Services:Introduced CM/SW role in discharge planning to assist with needs post-discharge including coordination to the appropriate level of care; discussed and provided options and choices accordingly that meets patients needs and goals, Freedom of Choice Disclosure, Need to verify with pts managed care organization for providers that are in-network
Education Provided to Patient Only
Teaching Method Verbal Instructions, Provided Community Resources
Education Outcome Verbalized Understanding
Discharge Plan
Anticipated Discharge Needs Home Health-RN, Home Health-PT
Anticipated Transportation Upon Discharge Private Vehicle/Car
Anticipated Discharge Destination Shelter/Recuperative Care

Electronically signed[redacted]

[redacted]

[redacted]

Electronically signed on Date [redacted]
Electronically signed on Time

[redacted]